

Strabismus after optical correction

CLINICAL TYPE B | Focused decisions for M3-M5

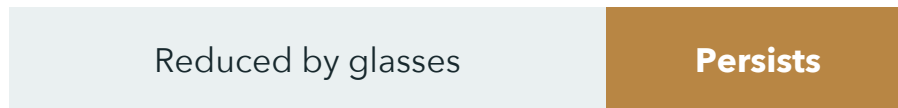
Separate accommodative and residual components

Spectacles and surgery act on different parts

UNCORRECTED DEVIATION: TWO COMPONENTS



FULL HYPEROPIC CORRECTION



Surgery targets the residual component

Glasses usually remain part of the plan

Original qualitative schematic; bar length is not a prism-angle scale.

Assess with the prescribed correction ON

Finding	Clinical inference
Aligned in glasses; crosses without them	An accommodative component remains controlled by the lenses.
Persistent stable crossing in glasses	Recheck cycloplegic refraction/adherence and assess the residual nonaccommodative component.
Substantial residual deviation	Specialist strabismus surgery may be considered while glasses continue.

Measurements answer different questions

Cover testing and function

Finding	Clinical inference
Cover-uncover	Look for a manifest deviation by observing refixation of the uncovered eye.
Alternating cover	Breaks fusion to reveal total deviation; measure with prisms as appropriate.
Near versus distance	Compare angles and binocular function; a near-predominant residual needs a different assessment from equal near/distance crossing.

Planning the next step

- 1 Check acuity in each eye and manage amblyopia alongside alignment.
- 2 Confirm the correct prescription, then repeat reliable measurements over time.
- 3 Explain that surgery treats the residual alignment problem and generally does not remove hyperopic spectacle need.